When it comes to Progesterone,

UTROGESTAN® *is the* ONE

micronised progesterone

Only 200mg Progesterone approved by FDA for vaginal route of administration [1]

(Also available for oral route: 200mg, 100mg)

Natural micronized progesterone (Innovator)

Effective in managing threatened miscarriage, preterm birth, luteal phase defect, and menstrual cycle irregularities[2,3,4,5]

REFERENCES:

1. FDA Verification Portal. (2021). Food And Drug Administration Philippines. https://verification.fda.gov.ph/drug_productslist.php?cmd=search&t=drug_products&psearch=progesterone&psearchtype
2. Czajkowski K, et al. Uteroplacental circulation early pregnancy complicated by threatened abortion supplemented with vaginal micronized progesterone or oral dydrogesterone. Fertility and Sterility. Vol 87, No. 3, March 2007: 613-618.
3. Romero R, et al. Vaginal progesterone in women with an asymptomatic sonographic short cervix in the midtrimester decreases pre-term delivery and neonatal morbidity: a systematic review and metaanalysis of individual patient data. American Journal of Obstetrics & Gynecology. 2012: 124.e1 – 124.e19
4. van de Linden M. Buckingham K, Farquhar C, Kremer JAM, Metwally M. Luteal phase support for assisted reproduction cycles. Cochrane Database of systematic reviews 2011, Issue 10
5. Bouzekrini M. Quality of life and Micronized Progesterone Cycle disorders in teenagers. 2006. Oral Communication.

OESTROGEL

BESINS HEALTHCARE
Innovating for Well-being

When it comes to Progesterone, UTROGESTAN® micronised progesterone *is the* ONE

UTROGESTAN® micronised progesterone	VS.	Brand H
Natural Micronized Progesterone (Innovator)	**Molecule**	**Natural Micronized Progesterone** (Generic)
Soft-gel capsule	**Form**	**Soft-gel capsule**
200mg | 100mg	**Available Dosage**	**200mg ONLY**
Vaginal (200mg) **Oral** (200mg | 100mg)	**FDA-Approved Route of Administration**	**Oral Administration Only**
Belgium / Thailand	**Manufacturing**	**India**

ONLY FDA-APPROVED 200mg PROGESTERONE FOR VAGINAL ROUTE

PRISM Progesterone In Sponteneous Miscarriage

The role of Utrogestan in threatened miscarriage
(enrichment via history of current pregnancy bleeding)

PROMISE
PROgesterone in recurrent MIScarriagE trial

The role of Utrogestan in recurrent miscarriage
(enrichment via history of previous miscarriages)

Major Clinical Trials

References being used are based on trials of the innovator brand (Utrogestan)

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When it comes to Progesterone, UTROGESTAN® micronised progesterone *is the* ONE

UTROGESTAN® micronised progesterone	VS.	Brand D
Natural Micronized Progesterone (Innovator)	**Molecule**	**Synthetic Progestogen** (Dydrogesterone)
Soft-gel capsule	**Form**	**Film-coated tablet**
200mg | 100mg	**Available Dosage**	**10mg**
Vaginal (200mg) **Oral** (200mg | 100mg)	**FDA-Approved Route of Administration**	**Oral Administration Only**

ONLY FDA-APPROVED 200mg PROGESTERONE FOR VAGINAL ROUTE

Safety

Neonatal effects, infant health and cost-effectiveness of Vaginal Micronized Progesterone capsules are now available with a level 1 of evidence

based on the following trials:

PREDICT (Rode et al)
FMF (Fenseca et al)
PREGNANT (Hassan et al)
OPPTIMUM (Norman et al)
EVENTS (Rehal et al)
PROMISE (Coomarasamy et al)
PRISM (Coomarasamy et al)

With **Dydrogestrone exposure,** there were **increased teratogenic risks** for congenital aortic insufficiency and overall cardiac malformations, as well as for hypospadias,spine bifida, and congenital hyrocephalus.

(Koren G. et al. Expert Rev Clin Pharmacol 2020)

When **Dydrogestrone was combined with IVF/ART, there was also increased risk for crytorchidism, hip dislocation and renal dyslasia**

(Koren G. et al. Expert Rev Clin Pharmacol 2020)

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When it comes to Progesterone, UTROGESTAN® micronised progesterone is the ONE

Use of oral progestogen in women with threatened miscarriage in the first trimester: a randomized double-blind controlled trial

Diana Man Ka Chan[1,*], Ka Wang Cheung[1], Jennifer Ka Yee Ko[1], Sofie Shuk Fei Yung[1], Shui Fan Lai[2], Mei Ting Lam[2], Dorothy Yuet Tao Ng[3], Vivian Chi Yan Lee[1], Raymond Hang Wun Li[1], and Ernest Hung Yu Ng[1]

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Submitted on July 26, 2020; resubmitted on October 19, 2020; editorial decision on November 2, 2020

STUDY QUESTION: Will use of oral progestogen in women with threatened miscarriage in the first trimester reduce the miscarriage rate when compared with placebo?

PARTICIPANTS/MATERIALS, SETTING, METHODS: Women attending Early Pregnancy Assessment Clinics because of vaginal bleeding during the first trimester were recruited and randomly assigned to use **Dydrogesterone (Duphaston)** 40 mg orally, followed by 10 mg orally three times a day or placebo until 12 completed weeks of gestation or 1 week after the bleeding stopped, whichever was later. The primary outcome was the miscarriage rate before 20 weeks of gestation.

SUMMARY ANSWER: Use of oral progestogen in women with threatened miscarriage in the first trimester **did not reduce miscarriage before 20 weeks when compared with placebo.**

WIDER IMPLICATIONS OF THE FINDINGS: **Use of oral progestogen is not recommended in women with threatened miscarriage in the first trimester.**

REFERENCES:

1. Use of oral progestogen in women with threatened miscarriage in the first trimester: a randomized double-blind controlled trial: Diana Man Ka Chan, Ka Wang Cheung, Jennifer Ka Yee Ko, Sofie Shuk Fei Yung, Shui Fan Lai, Mei Ting Lam, Dorothy Yuet Tao Ng, Vivian Chi Yan Lee, Raymond Hang Wun Li, and Ernest Hung Yu Ng
2. Human Reproduction, Vol.00, No.0, pp. 1-9, 2020 doi: 10.1093 / humrep/ deaa327

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A Spectrum of Hope in Saving Lives

PRISM Progesterone In Spontaneous Miscarrage

The effectiveness of progesterone to prevent miscarriage in women with early pregancy bleeding

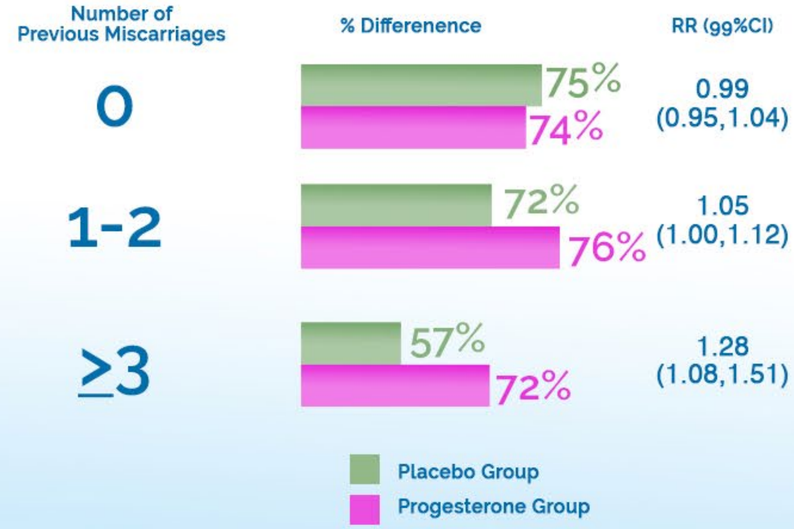
A randomized placebo-controlled trial

POPULATION: Women with early pregnancy bleeding during first 12 weeks of pregnancy

INTERVENTION: 400mg of micronized progesterone (Utrogestan®) taken vaginally twice daily from the time of bleeding presentation until 16 weeks of pregnancy

PRIMARY OUTCOME: Live births ≥34 weeks

THE PRISM Trial Subgroup Analysis[1]



Women with a history of miscarriage may benefit from the use of vaginal micronized progestone (Utrogestan®) 400mg twice daily

REFERENCE: Coomarasamy A, et al. N Engl J Med 2019; 380: 1815-24

NICE National Institute for Health and Care Exc

NICE guideline

1.5 Management of miscarriage

Threatened miscarriage

1.5.2 Offer vaginal micronised progesterone 400 mg twice daily to women with an intrauterine pregnancy confirmed by a scan, if they have vaginal bleeding and have previously had a miscarriage. [2021]

1.5.3 If a fetal heartbeat is confirmed, continue progesterone until 16 completed weeks of pregnancy. [2021]

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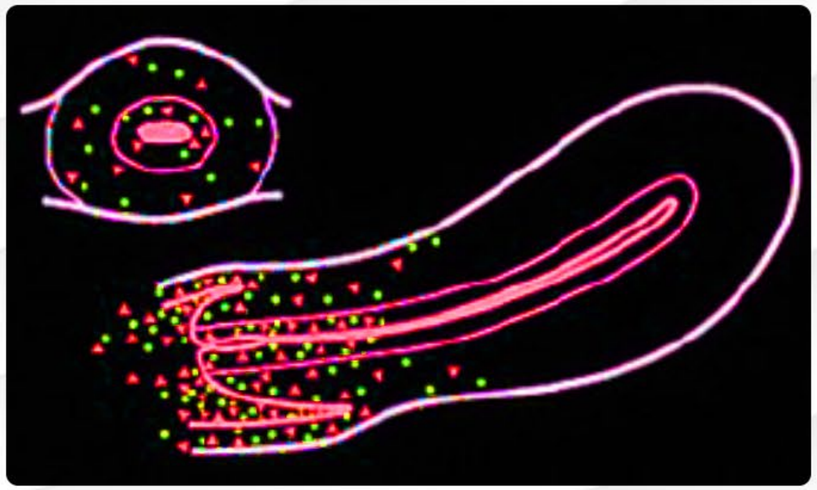
Vaginally administered progesterone:
THE PREFERRED ROUTE

- First uterine pass effect
- Better bioavailability of progesterone in uterus
- Achieves adequate endometrial secretory transformation
- Minimal systemic undesirable effects

Autoradiography of human uterus obtained after vaginal application of radioactive progesterone (using human ex-vivo uterine perfusion model)



1 hour after vaginal application

4 hours after vaginal application

▲ Radioactive reference substance

● Radioactive progesterone

REFERENCES:

1. Tavaniotou A, et al. Hum Reprod Update. 2000 Mar-Apr;6(2): 139-48.
2. Bulletti C, et al. Hum Reprod. 1997 May; 12(5):1073-9.

You have the freedom in the route of administration

UTROGESTAN® micronised progesterone

Your natural partner against THREATENED ABORTION

VAGINAL or ORAL

200mg - 400mg BID until 12th week o f pregnancy

UTROGESTAN® micronised progesterone

Your natural partner against PRETERM LABOR

VAGINAL ADMINISTRATION	
Acute Phase	200mg-300mg BID/TID
Maintenance until 36th week	100mg-200mg/day

ORAL ADMINISTRATION	
Acute Phase	400mg every 6-8 hours
Maintenance until 36th week	200mg-300mg/ OD or BID

UTROGESTAN® micronised progesterone

Your natural partner in INFERTILITY and IVF

INFERTILITY ASSOCIATED WITH LUTEAL PHASE DEFECT	
Initial dose on days 13 & 14 of the transfer cycle	100mg/OD
Days 15 to 25	100mg/BID
From Day 26-60, in the event of conception, dosage is increased in increments of 100mg/week to reach a maximum dose of 600mg per day, spread over three doses	600mg (maximum)

LUTEAL PHASE SUPPORT IN IVF INITIATED IN THE EVENING OF TRANFER
600mg spread over three doses

VAGINAL

ORAL

BESINS HEALTHCARE
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Choose the **ONE** *and let life continue to bloom with* Œstrogel®

ESTRADIOL-17ß

Only FDA-approved transdermal Estradiol available in the market [1]

Not likely to produce thrombotic risk, stroke and coronary artery disease [2]

Effective in managing vasomotor symtoms (VMS) and genitourinary syndrome of menopause (GSM) [3]

REFERENCES:

1. FDA Verification Portal. (2021). Food And Drug Administration Philippines. https://verification.fda.gov.ph/drug_productsview.php?showdetail=&registration_number=DR-XY42910
2. Cobin RH, Goodman NF on behalf of the AACE Committee. Endocrine Practice 2017; 23(7): 869-880
3. NAMS 2017 Position Statement. Menopause 2017; 24(7): 728-753

HOW TO USE Œstrogel® ?

ESTRADIOL-17ß

1

Open the cap and use perforator to unseal the tube.

2

Squeeze out the gel on the spatula evenly. The usual starting dose is a single spatula measure (2.5g) once daily.

3

Gently spread the gel to clean, dry, intact areas of the skin (e.g. on the abdomen, arms, shoulders, and/or inner thighs). The area of application should be as large as possible.

4

Allow the gel to dry for 2 minutes before covering the skin with clothing.

Gel does NOT need to be massaged into skin

Gel must NOT be applied on the breasts or mucous surfaces such as the vulval region

*If the patient forgets to apply a dose, then she should apply it as soon as possible and apply the next dose at the normal time. Forgetting a dose may increase the likelihood of break-through bleeding and spotting.

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